

VALLEY GENERAL HOSPITAL

Sahasra plaza near geethabhavan, kanchit chowrastha ,Mukarampura, karimnagar, 505001

PATIENT MEDICAL REPORT

CHIEF COMPLAINT & HISTORY OF PRESENT ILLNESS Patient is a 64-year-old male with a past medical history of hypertension and hyperlipidemia, who presents with acute onset of substernal chest pain, described as "pressure," which began approximately 2 hours ago while watching television. The pain radiates to his left arm and is associated with mild shortness of breath and diaphoresis. The pain was not relieved by rest. He denies any nausea, vomiting, or syncope.

PAST MEDICAL HISTORY

Hypertension (HTN)

Hyperlipidemia (HLD)

Type 2 Diabetes Mellitus (T2DM), diagnosed 5 years ago, diet-controlled.

CURRENT MEDICATIONS

Lisinopril 10mg daily

Atorvastatin 20mg daily

Aspirin 81mg daily

ALLERGIES

No Known Drug Allergies (NKDA)

SOCIAL HISTORY

Former smoker (quit 10 years ago, 20 pack-year history).

Occasional alcohol use (1-2 glasses of wine per week).

Works as an accountant. Lives with wife.

FAMILY HISTORY Father had a myocardial infarction at age 55. Mother has hypertension.

REVIEW OF SYSTEMS Constitutional: Denies fever or weight loss. Cardiovascular: As per HPI. Denies palpitations or edema. Respiratory: As per HPI. All other systems reviewed and are negative.

PHYSICAL EXAMINATION

Vitals: BP 165/95, HR 102 bpm, RR 18, SpO2 96% on room air, Temp 37.0°C.

General: Patient appears anxious and in moderate distress due to pain.

Cardiovascular: Tachycardic rate, regular rhythm. No murmurs, rubs, or gallops.

Respiratory: Lungs clear to auscultation bilaterally, no wheezes or rales.

Abdomen: Soft, non-tender, non-distended. Bowel sounds present.

DIAGNOSTIC RESULTS

ECG: Sinus tachycardia with ST-segment elevation of 2mm in leads V2-V4, consistent with an anterior STEMI.

Labs: Elevated Troponin I at 8.5 ng/L (ref: <0.04). CBC and Basic Metabolic Panel within normal limits.

Chest X-Ray: No acute cardiopulmonary findings.

ASSESSMENT & IMPRESSION

Acute Anterior ST-Elevation Myocardial Infarction (STEMI): Primary cause of presentation, confirmed by ECG and elevated cardiac enzymes.

Hypertension, uncontrolled.

Hyperlipidemia.

Type 2 Diabetes Mellitus.